



Patient Name : MOHAN T
Age/ Sex : 75 YEARS/MALE
Adm Date : 1ST JUNE 2026
Disc Date : 4TH JUNE 2026
Address : ROOPA NAGAR, ALANAHALLI, MYSORE
Hospital No : 30894
IP No : 797
Bed Number : DELUXE
Consultant : DR. RAJAGOPAL J
Specialty : CARDIOLOGY

DISCHARGE SUMMARY

DIAGNOSES:

- Acute LVF
- Double vessel coronary artery disease of LAD & RCA
- Mild left ventricular systolic dysfunction, EF 45%
- Type II Diabetes Mellitus
- Hypertension
- AKI

HISTORY: Patient is a known case of diabetes, hypertension, CAD-old AWMI. CAG done in 2016 showed LAD (CTO), collaterals form RCA. Stress thallium showed no inducible ischemia with LAD scar and hence on medical management. Presented with history of breathlessness on minimal exertion since few days.
H/O Severe generalized weakness.
No H/O Chest pain, fever, vomiting, palpitation.

On Examination:

Pulse - 94/min
BP - 130/70mmHg
CVS - S1 S2 normal
RS - B/L basal mid crepts +
P/A - Soft
CNS - NAD

INVESTIGATION DONE: Reports enclosed

ECG : SR, normal axis, ST depression in II, III, aVF, V1-V6

2D ECHO: 01.06.2026

Ischemic heart disease
Hypokinetic septum, apex, basal inferior and basal posterior wall
Concentric left ventricular hypertrophy
Mild left ventricular systolic dysfunction, EF 45%
Grade II LV diastolic dysfunction
Mild MR and TR
Normal RV function
No clot. No pericardial effusion
IVC normal collapsing

CAG: 03.06.2026

Double vessel coronary artery disease of LAD & RCA

avant bkg hospitals

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Mysore - 570 026
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Aceso BKG Health Care

Near Vijaya Bank Circle,
Kuvempunagar, Mysore - 570 023
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avant bkg health care

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08232 227272 | +91 73497 27272

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COURSE IN THE HOSPITAL: Patient was admitted with above mentioned complaints. His investigation revealed normal CBC, Trop I Negative. Creatinine 1.8. 2D Echo showed Mild left ventricular systolic dysfunction, EF 45%. Patient was managed with Nikoran infusion, statins, beta blockers, diuretics, antibiotics and other supportive measures. Patient symptomatically better and planned for CAG. CAG was done on 3rd June 2026 which revealed Double vessel coronary artery disease of LAD & RCA and advised Stress MRI to decide on revascularization. Post procedure patient was stable. Patient is being discharged with stable haemodynamics.

At the time of discharge: HR- 70/min

BP-120/70mmHg

ADVISE ON DISCHARGE:

LOW FAT, LOW SALT DIET

1. TAB. NOVASTAT CV	20MG	0-0-1
2. TAB. CARDIVAS	3.125MG	1-0-1
3. TAB. OXRA	10MG	1-0-0
4. TAB. K-COR	5MG	1-0-1
5. TAB. DYTOR	10MG	½ -0-0
6. TAB. NEFORSAVE		0-0-1
7. TAB. NODOSIS	500MG	1-0-1
8. TAB. TAXIM O	200MG	1-0-1 X 5 DAYS
9. TAB. PANTOCID	40MG	1-0-0 X 7 DAYS (BEFORE FOOD)
10. TAB. SORBITRATE	5MG S/L	SOS
11. TAB. MUCINAC	600MG	1-0-1 X 5 DAYS

Review in Cardiology OPD after 1 week with RFT report.


Dr. Rajagopal J
Chief consultant Cardiologist

Dr. Suhas Alur
Consultant Cardiologist

Dr. Vanita S Bhat
Clinical Cardiologist

*For emergency please contact: 0821-2520000: Cardiology Helpline-9901752525
(In case patient has chest pain, breathlessness, giddiness or bleeding)*